A&E four-hour performance

Forecast for October 2026 · data through September 2026 · issued 2026-10-08

National all-types performance fell by 0.7pp over the three months to September 2026, reaching 74.4%. The October 2026 forecast is 74.9%, 0.5pp above the latest result. The model's expected improvement is mainly driven by the underlying trend; the seasonal effect is small. The reference projection reaches 76.9% by March 2027, 5.1pp below the 82% interim milestone.

LATEST · SEPTEMBER 2026	FORECAST · OCTOBER 2026	ACTUAL
74.4%	**74.9%**	—
Known when the forecast was made	80% expected range 73.0%–76.7%	The published result will be added here without removing the forecast.
		10.1pp below the 85% 2028/29 milestone

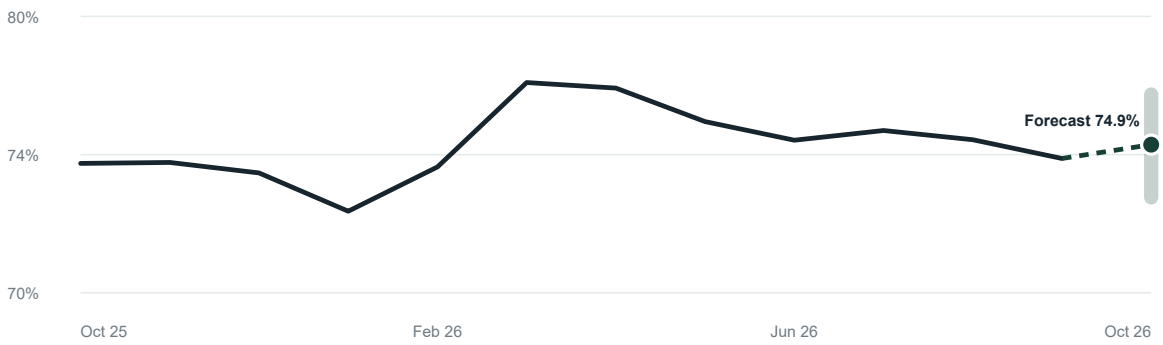
WHAT CHANGED?

Demand was higher than a year earlier and performance weakened: the pattern is consistent with demand rising faster than processing and flow capacity.

Attendances	Seen within 4 hours	Breaches
5.8%	4.9%	8.7%

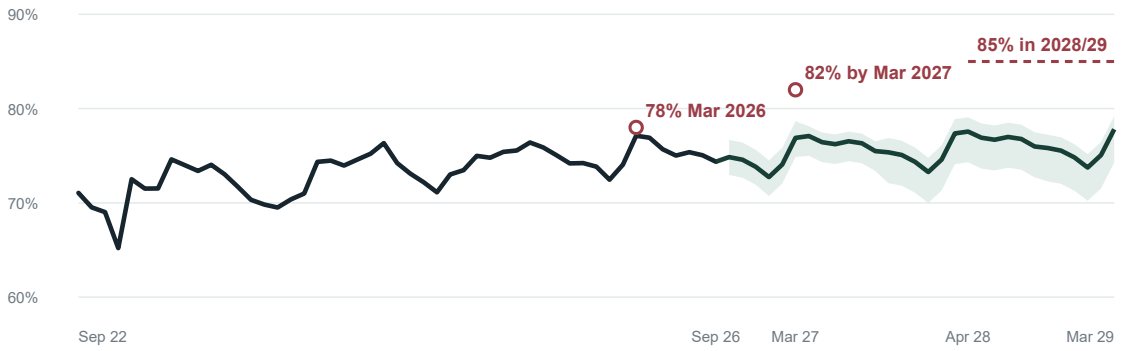
Recent performance and next release

The vertical band is the empirical 80% expected range.



Longer-term outlook and milestones

Descriptive projection; not forced to meet targets.



Benchmark: 95% constitutional standard; planning milestones shown on the chart.

Data: NHS England A&E attendances and emergency admissions. **Targets:** Medium Term Planning Framework.

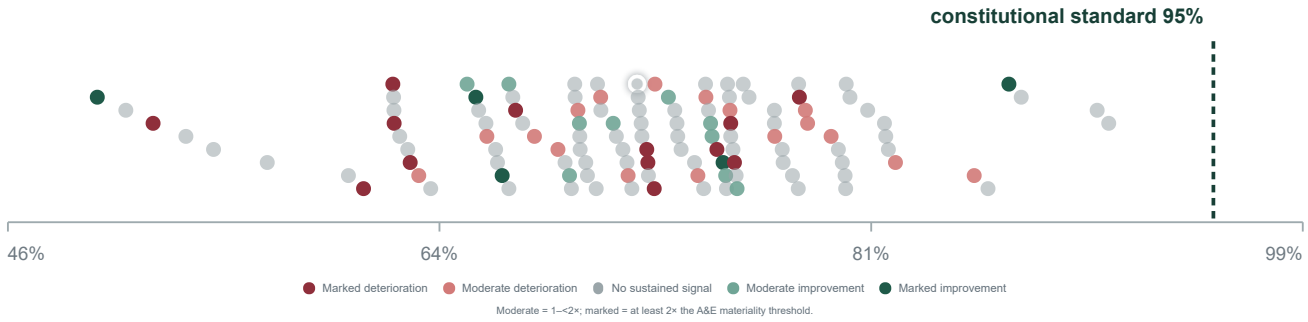
Method: Fixed equal-weight ensemble of same-month persistence, damped annual drift and a shared-season current-era trend; all three components are required. The final fit uses 191 months. Accuracy is tested on 16 rolling one-month-ahead forecasts. The 80% range uses 81 empirical current-era forecast errors. The pre-release commentary compares the seasonal component with the two trend-bearing components. Provider distribution and trajectory watch are on page 2.

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Distribution and trajectory watch · September 2026

Provider distribution

Latest reported performance across providers. This is a comparison of current levels, not a measure of whether each provider is improving or deteriorating.



Moderate = 1–<2×; marked = at least 2× the A&E materiality threshold.

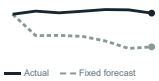
Trajectory watch

Six actual months are compared with one forecast path fixed before the window. Both lines begin at the same observed value. The displayed gap is the latest actual minus the fixed forecast for that same month. A low performer can be improving, while a high performer can be deteriorating.

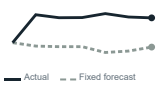
120	15	30	75
providers assessed	sustained favourable	sustained adverse	without an active signal

Sustained adverse signals outnumber favourable signals (30 versus 15). Most providers (75 of 120) do not meet the active six-release threshold.

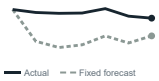
Largest sustained favourable moves

Nottingham University Hospitals NHS Trust
Latest 66.2% · 6 of 6 months in this direction · Active
Forecast fixed before the six-month window


+12.4pp
LATEST MONTH: ABOVE FORECAST

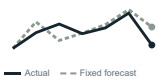
North Cheshire And Mersey NHS Foundation Trust
Latest 75.2% · 6 of 6 months in this direction · Active
Forecast fixed before the six-month window


+10.8pp
LATEST MONTH: ABOVE FORECAST

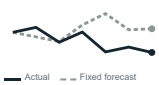
George Eliot Hospital NHS Trust
Latest 86.7% · 6 of 6 months in this direction · Active
Forecast fixed before the six-month window


+8.8pp
LATEST MONTH: ABOVE FORECAST

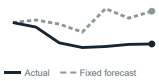
Largest sustained adverse moves

Birmingham Women's And Children's NHS Foundation Trust
Latest 78.2% · 5 of 6 months in this direction · Active
Forecast fixed before the six-month window


-8.7pp
LATEST MONTH: BELOW FORECAST

North West Anglia NHS Foundation Trust
Latest 66.8% · 5 of 6 months in this direction · Active
Forecast fixed before the six-month window


-6.4pp
LATEST MONTH: BELOW FORECAST

Royal Cornwall Hospitals NHS Trust
Latest 75.5% · 6 of 6 months in this direction · Active
Forecast fixed before the six-month window


-6.3pp
LATEST MONTH: BELOW FORECAST

Trajectory-watch rule. The latest 6 actual months are compared with a path fixed before that window. The displayed gap is the latest actual minus the fixed forecast for that same month. Its magnitude must be at least 2.0pp, and at least 5 of 6 months must point in that direction. These are screening prompts—not league tables, causal estimates or judgements of provider quality.